



Speech Pathology: Outpatient Services and Administration Tasks Procedure

1. Scope

Site	Operational Area	Applicable to
Armadale Kalamunda Group	Speech Pathology Department	Speech Pathology

2. Purpose

To outline the administrative tasks required to receive, manage and discharge outpatient referrals to the Armadale Health Service (AHS) Speech Pathology (SP) Outpatient (OP) service.

The SP OP service provides intervention to patients requiring assessment, treatment and management of speech and language disorders, swallowing, acquired voice and fluency disorders.

3. Procedure

3.1 Eligibility

To be eligible, patients must live in the Armadale Health Service catchment area as per the Catchment area post codes.

3.2 Referral procedure

- Referrals are to be completed via e-referrals or on the East Metropolitan Health Service Allied Health and Community Rehabilitation referral form:
<http://www.ahs.health.wa.gov.au/~media/Files/Hospitals/AHS/Referral%20forms/community-rehabilitation-referral-form.pdf>
- Referrals are triaged by the Speech Pathology Coordinator as per Appendix 1: Visual Referral Process.
 - Referrals are booked by the outpatient clerk.

3.3 Managing individual patients

See Appendix 2: SP Non-Admitted Appointment process and Appendix 3: Patients with Progressive Neurological Disorders

3.4 Phone reviews

- Phone reviews are generally booked retrospectively by the managing clinician (however some may be planned in advance)
- Complete the procedure as per Appendix 2
- Complete details on the clinic list including the time the phone call was made and hand to clerk at the end of the day

3.5 Videofluoroscopy patients

- Patients referred only for videofluoroscopy swallow studies (VFSS) are required to be seen for a 1:1 assessment with a SP prior to booking the VFSS. If referral from external SP no initial consult required.
- When an aged care facility patient, referral from private clinician or NDIS participant is referred for a VFSS, it is the responsibility of the Coordinator to liaise with the treating SP to discuss appropriateness and suitability for a VFSS.
- For aged care facility or private and NDIS participants, it is preferred that they attend the VFSS with their managing SP or a nurse/carers from their facility.
- AHS does not arrange transport for aged care facility patients attending VFSS
- Refer to [AKG Videofluoroscopy Swallow Study Procedure](#) for specific VFSS procedures

3.6 Discharge – Outpatients only

- Due to the high demand for outpatient services, it is essential that referrals and appointments are managed in accordance with the [OD0530/14 - Specialist Outpatient Services Access Policy \(pdf 374KB\)](#) and the [AKG Outpatient Referral Triage and Management Guidelines](#)

3.6.1 Did Not Attend (DNA)

- To ensure an equitable and timely service for all patients, patients who do not attend their clinic appointment without notifying the hospital prior will be classified as “did not attend” (DNA) appointment.
- A patient will be discharged or removed from the clinic appointment list following the patient’s first DNA for an initial appointment or their second consecutive DNA for a follow up appointment in the same clinic, unless prior notice or good cause is provided to the hospital.
- Clinicians must exercise discretion on a case-by-case basis to avoid disadvantaging patients in the case of genuine hardship, high risk factors, misunderstanding and other unavoidable circumstances when discharging patients.
- The patient and General practitioner must receive written notification of their discharge and removal from the clinic appointment list.

3.6.2 Patient cancellation

- Although it is preferable for patients to provide adequate notice of non attendance or cancellation of their outpatient appointment, it is recognised that circumstances beyond the patient’s control may make this impracticable. Therefore, an appointment may be classified as cancelled up to the time of the appointment at the discretion of the clinician.
- Where a patient attempts to cancel the appointment after the scheduled time, the appointment will be classified as a DNA appointment and the reason for non-attendance to be recorded.
- Patients who repeatedly cancel and defers appointments on 2 consecutive occasions for the same clinic will be discharged.
- If the patient has indicated non-availability for a period of 90 days or more without prior notice and provision of a good cause will be discharged.
- The patient and General Practitioner must receive written notification of their discharge and removal from the clinic appointment list.

3.6.3 Patient discharge

In addition, a patient may be discharged under the following circumstances:

- Patient is receiving treatment for the same condition at another hospital.
- Patient no longer wishes to receive treatment at their own choice.

The discharge of a patient should be clearly documented in the patient's medical record with a recommended plan for the GP, should the patient require a re-referral.

4. Roles and responsibilities

Role	Responsibility
Speech Pathology Coordinator	Triaging / allocating priority for referrals
Outpatient clerk	Booking appointment, obtaining hardcopy medical files if required, rebooking appointments
Speech Pathologist	Assessment and treatment, documentation, specifying management plan

5. Legislative context

The following documents are required to give effect to this procedure [i.e. the documents included are mandatory]:

- [WA Health Consent to Treatment Policy](#)
- [East Metropolitan Health Service \(EMHS\) Consent Policy](#)

6. Supporting information

The following documents support the implementation of this procedure:

- [Videofluoroscopy Swallow Study Procedure AKG-PRO-0464-18](#)

7. Compliance, monitoring and evaluation

The Coordinator of Speech Pathology will be responsible for the compliance of this document. Receipt of consumer complaints and local speech pathology documentation audit processes will be used to review trends and investigate incidents on an as needs basis/as required and to identify any opportunities for improvement.

8. Relevant standards

National Safety and Quality Health Service (NSQHS) Standards (second edition):

- Standard 5 – Action 5.4: The health service organisation has systems for comprehensive care that:
 - a. Support clinicians to develop, document and communicate comprehensive care plans for patients' care and treatment
 - b. Provide care to patients in the setting that best meets their clinical needs
 - c. Ensure timely referral of patients with specialist healthcare needs to relevant services

- d. Identify, at all times, the clinician with the overall accountability for a patient's care

9. Policy owner (relating to these procedures)

Coordinator Speech Pathology

Enquiries relating to this procedure may be directed to:

Title: Coordinator Speech Pathology
 Department: Speech Pathology
 Email: Ashleigh.Vidovich@health.wa.gov.au

10. Review

This procedure will be reviewed and evaluated as required to ensure relevance and currency. At a minimum it will be reviewed within one (1) year after first issue and at least every three (3) years thereafter.

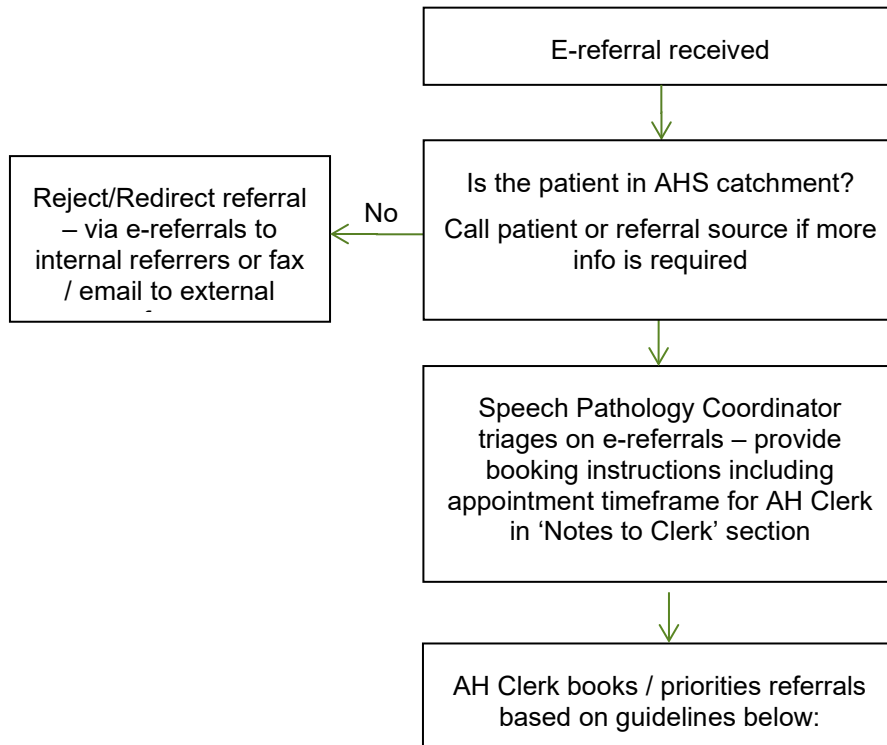
Version	Effective from	Effective to	Amendment(s)
V3.1	23/06/2025	15/11/2027	Section 3.2 Referral Procedure: Referrals are triaged by the Speech Pathology Coordinator Section 3.3 and 3.4 minor administrative amendments Section 3.6 – Discharge – Outpatients only Due to the high demand for outpatient services, it is essential that referrals and appointments are managed in accordance with the OD0530/14 - Specialist Outpatient Services Access Policy (pdf 374KB) and the AKG Outpatient Referral Triage and Management Guidelines” • 3.6.1 Did not attend information • 3.6.2 Patient cancellation information • 3.6.3 Patient discharge information

11. Authorisation

This procedure has been authorised and issued by the Director Allied Health and Ambulatory Care.

Authorised by	Danielle Kilmurray – Director Allied Health and Ambulatory Care
Authorisation date	20/06/2025
Published date	23/06/2025

Appendix 1: Visual Referral Process for Outpatient Speech Pathology Referrals (2025)



- **Allied Health Clerical Procedure** – Referrals awaiting booking will appear on WebPas worklist once processed by Speech Pathology Coordinator. Book as per instructions given in Notes to Clerk. Contact patient to make appointment. If required, the clerk may order the hardcopy medical file on request.
- **For Videofluoroscopy Referrals** - AHS Speech Pathologist / Coordinator will determine if an assessment is required prior to progressing to VFSS. AHS Speech Pathologist / Coordinator will book VFSS appointment.

Speech Pathology Clinicians (Basic administration responsibilities for appointments)

- Clinician conducts initial appointment.
- Clinician to complete review timeframe or d/c on the clinic list
- Clinician to inform patient to report to bookings clerk following appointment.
- Clinician to tick or cross appointment / time sheet printout to indicate attendance or DNA
- For phone reviews, clinician to record on the clinic list.

**For further details please see Department specific policies and procedures.

CR Groups – If an outpatient Occupational Therapy or Speech or Dietetics patient is suitable for a CR group; a referral to the specific CR group needs to be completed via e-referral to Rehab Medicine.

Appendix 2: Managing individual patients – day of appointment

Medical Records

- If a hardcopy medical record is required and requested, it will be left at the CR reception.

Appointment bookings

- The outpatient clerk is responsible for booking appointments and corresponding with the patient
- Hospital transport is not available for outpatients

Day of Appointment

- Provide the appropriate session
- At the end of the session, if the patient requires ongoing SP input, provide them with an appointment outcome slip to present to the clerk or inform patient of review plan, with an appointment letter to be sent to them. Provide details regarding when the patient will require another appointment and how many appointments are needed (i.e. next available, 6 weeks, 5 weekly sessions.)
- If the patient does not require a review for 3 or more months, an appointment outcome slip is not completed. The managing SP instead, completes 'action' on Clinic Appointment List (eg face to face review 6 months).
- Document in the patient's medical record, include copies of any reports/correspondence etc. Assessment materials cannot be uploaded in the digital medical records, so ensure that any raw data is summarised in a formal report or within the progress notes
- Summarise all care and outcome on iSoft
- If a hardcopy medical record was obtained, return medical record to the clerical team; place in the blue returns trolley for CR, or on the medical records shelf for OP
- At the end of the day, return the patient list, ensuring to mark if patients attended, did not attend (DNA), cancelled their appointment or are for discharge
- *Initial assessments:*
 - Decide on the type and duration of services to be offered to the patient
 - Provide a letter to the referring agency detailing the findings, recommendations and plan for review/discharge
- *Patients on regular review:*
 - Decide how frequently the patient requires review. Refer to [Appendix 3: Patients with Progressive Neurological Conditions Procedure](#)

Appendix 3: Patients with Progressive Neurological Conditions – SP management

This management guide applies to all patients with progressive neurological conditions including but are not limited to Parkinson's disease (including Multiple System Atrophy (MSA), Progressive Supranuclear Palsy (PSP) and Lewy Body Dementia (LBD), Huntington's disease, Multiple Sclerosis, and Motor Neurone Disease.

1. Patients will be reviewed at least every 6 months with either a telephone or face to face review. This is at the discretion of the patient's managing SP. A telephone review is suitable for patients who are not currently having difficulties with communication or swallowing. However, all patients with progressive neurological conditions require a face-to-face review at least every 12 months. Appointments can then be scheduled as required. If patients require more than one allied health discipline, then it is preferable to transfer the patient to Community Rehabilitation.

2. Inpatients with progressive neurological conditions will be transferred to the non-admitted patient caseload (with their consent) upon discharge from the ward, by the treating clinician and reviewed within 6 months of discharge. Inpatients residing outside the Armadale catchment area will be transferred to the appropriate Speech Pathology Department for follow-up (see Clinical Handover policy for more information)

- Inpatient clinicians are to submit an e-referral for 6months or less for follow up appointment post discharge

3. Patients with progressive neurological conditions are not discharged from the Speech Pathology Caseload, unless they no longer meet eligibility requirements, e.g. transferred to permanent residential care.

4. Community Rehab 2 (CR2) patients with Parkinson's disease will remain with CR2 (i.e. will not be transferred to OP SP). See CR2 policy re: Parkinson's patients for details [Community Rehabilitation Triage Process for Parkinsons Clinic](#)

NOTE: If patients with progressive neurological conditions cannot be contacted by phone to arrange their routine appointment, then a letter may be sent to notify them that they are overdue for review.